Everolimus + exemestane

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Everolimus + exemestane · supportive-care plan

Active medicines: Everolimus + Exemestane

Emetogenic risk: Oral minimal–low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Everolimus is oral minimal–low emetic risk. No routine infusion premedication is needed; agree individual rescue treatment and local oral-systemic anticancer therapy (SACT) advice.
- Agree non-alcoholic mouth care and whether a prescribed alcohol-free corticosteroid mouthwash is appropriate during the first eight weeks. Report mouth ulcers early and check for thrush rather than adding an antifungal automatically.
- New cough, breathlessness or fever needs prompt review. If systemic steroids or other immunosuppression are used, the team considers Pneumocystis jirovecii pneumonia (PJP) prevention according to risk.
- Keep glucose, cholesterol, kidney and liver checks. Discuss diabetes/lipid care, exemestane bone risk, exercise, diet and any indicated bone-protection treatment.
- Tell both oncology and procedure teams about everolimus before surgery or radiotherapy. Check all new medicines and supplements; avoid grapefruit and St John’s wort.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00322 v4 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required